



PROCEDURE

Admission to CAMHS Inpatient Service

Scope (Staff):	All CAMHS Inpatient Service staff
Scope (Area):	CAMHS Ward 5A and Nickoll Ward

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations.

Read the full statement here:

[CAHS Child Safe Organisation Commitment Statement](#)

This document should be read in conjunction with this [disclaimer](#)

Contents

Aim	2
Risk.....	2
Background.....	2
Definitions	2
Principles	3
Cultural Considerations.....	3
Admission Criteria.....	3
Internal referrals from PCH Emergency Department	3
Internal referrals from PCH Paediatric Ward.....	4
External Referrals.....	4
Planned Admission Requests	5
Preadmission Meeting	6
Patient Registration.....	7
Welcome to Ward Meeting and Start of Assessments.....	7
Ward Orientation.....	9
Consent and Assessments	10
Documentation.....	11

Aim

To provide guidance to staff regarding admission to CAMHS Inpatient Service. This procedure is consistent with the requirements in the [Mental Health Act 2014 \(WA\)](#) (MHA) and MP 0164/21 [Patient Activity Data Policy](#).

Risk

The most appropriate individualised clinical care is not provided for the child.

Non-compliance with the MHA, including risk of legislated penalties.

Background

This procedure outlines steps from the initial referral to assessments conducted during the first 24 hours. It includes clinical measures and data collection during admission.

The CAMHS Crisis Connect (CCC) Clinical Nurse Consultant (CNC) is the single point of access to the Ward 5A and patient flow coordinator. Decision as to whether a child will be admitted is made by the CCC Consultant Psychiatrist (or delegate) in consultation with the CCC CNC. Consideration is given to ward milieu and priorities on the waitlist determined by mental state, risk and level of community support available.

Ward milieu and patient mix is a critical factor in providing a safe environment for young people who require admission. The psychiatrist may decline any admission if they assess the risk to others as being unacceptably high.

Definitions

Admitted patient: A child that is admitted to CAMHS Inpatient Service following assessment against the CAMHS Inpatient Service admission criteria. The child must undergo a formal documented admission process to receive inpatient treatment and/or care for a period of time (see MP 0164/21 [Patient Activity Data Policy](#)).

Child: An infant, child, adolescent or young person under the age of 16 years (used as it aligns with age eligibility at Perth Children's Hospital (PCH)). See [Admission and Age of Eligibility Policy](#).

Family: The person(s) who has parental responsibility for the child. Parental responsibility, in relation to a child, means all the duties, powers, responsibilities and authority which, by law, parents have in relation to children (MHA s 4). For a child of Aboriginal or Torres Strait Islander descent, this may be a person from their kinship group who is recognised under customary laws and traditions as their carer (MHA s 281(2)(b)).

Nominated person: The person nominated under section 273(1) of the MHA. The role of a nominated person is to assist the child by making sure that the child's rights and interests are upheld. Nominated persons can be a friend or relative of the child or a child's guardian. It is whoever the child feels should receive information and who can help the patient. The person nominated must be an adult over the age of 18.

Principles

The principles from the [Charter of Mental Health Care Principles](#) (MHA sch 1), underpin this procedure. The principles also guide service delivery in CAMHS.

Cultural Considerations

The cultural and spiritual needs of all Aboriginal patients on CAMHS Inpatient Service must be considered as an integral part of their care planning. When an Aboriginal patient is admitted, the CAMHS Inpatient Service Aboriginal Mental Health Worker (AMHW) and the Aboriginal Mental Health Coordinator must be notified within 24 hours. If the patient is already engaged with a community-based AMHW, they should also be notified of this admission. Regular consultation with AMHWs must occur until the Aboriginal child or young person is discharged from CAMHS Inpatient Service.

Admission Criteria

Internal referrals from PCH Emergency Department

All referrals from PCH ED must be made following a mental health assessment by the CCC CNS or the psychiatry registrar in consultation with the on-call psychiatrist.

Once a decision to admit has been made, the child must be deemed fit for admission and must not require acute medical intervention.

Following decision to admit being endorsed by the CCC Consultant Psychiatrist, the CCC CNC (or proxy) will organise an Admission Huddle.

The Admission Huddle will be arranged via Vocera and held within 30 minutes to provide a verbal handover of the assessment. This meeting will be used to determine the POD allocation based on the POD Allocation for CAMHS Inpatient Service – Traffic Light Assessment Work Instruction.

Members of this meeting must include:

- Clinical Nurse Specialist (or proxy) from both Ward 5A and Nickoll Ward
- CCC clinician who conducted the assessment
- All parties are to be logged into their appropriate Vocera groups. At a minimum there must be one member of senior nursing staff from both sites in attendance.
- Admission Huddle occurs via MS Teams
- CCC provide a handover of their assessment and the senior nursing representatives decide whether the patient should be admitted to Ward 5A or Nickoll Ward
- CCC clinician to inform the On-Call Consultant Psychiatrist of the outcome and location for admission
- CCC clinician to document the huddle outcome as part of the bed request service event on PSOLIS

For young people who are being admitted internally via PCH Emergency Department whom have had an admission within the previous 12 weeks transfer to Ward 5A can occur following receipt of a cRAMP and detailed preadmission plan.

Internal referrals from PCH Paediatric Ward

All referrals from paediatric ward within PCH must be made following a mental health assessment by the PPIRT (Psychiatric Paediatric In-reach Team) or Eating Disorders Service clinical staff during business hours or the psychiatric registrar outside of business hours.

For young people admitted to Perth Children's Hospital (PCH) with complex medical needs, upon receipt of the required documentation, the CCC CNC (or proxy) will convene an urgent Pre-admission Huddle within 1 hour.

- Invitees to the Preadmission Planning Meeting include:
- Ward 5A Psychiatrist
- AMED/General Paediatrics Consultant
- PPIRT/EDS Psychiatrist
- CCC Clinical Nurse Consultant (or proxy)
- Ward 5A Clinical Nurse Specialist
- Medical Ward Clinical Nurse Specialist (or proxy)

A detailed medical and nursing plan must be documented in the patient's DMR and attached to the Preadmission Plan by the CCC CNC (or proxy).

External Referrals

Referrals from outside PCH may come from Metro/ WACHS CAMHS teams, private Psychiatrists, Authorised Mental Health Practitioners, other Emergency Departments, or medical wards at other hospitals.

The young person should be assessed by local CAMHS clinician/ private psychiatrist. All Community CAMHS referrals should be endorsed by the local Psychiatrist.

Referrals are made via the Ward 5A patient flow line (0466 419 342), or by email to camhscrisisconnect@health.wa.gov.au. For all urgent bed requests the referrer must contact the CCC CNC via telephone to discuss.

The referring clinician should provide assessments completed within the last 7 days.

Minimum Paperwork required must include the following in line with the Clinical Handover policy:

- SSCD Initial Assessment or more recent comprehensive assessment (in ISOBAR format)

- SS CD Risk Assessment
- Medical Clearance
- Rationale for admission
- Primary goals of admission

Planned admissions may be used as part of a Clinical Care Package. Referrers are to liaise with CCC. Clinical responsibility for the patient remains with the referrer until admission to Ward 5A.

A waitlist will be created on Enterprise Bed Management (EBM) when all Ward 5A beds are occupied and adjusted depending on clinical acuity. While awaiting admission the child's level of risk and the need for inpatient admission must be reviewed at least daily, and as clinically indicated, whilst the child is waitlisted. Where clinically indicated, the bed request may be escalated or revoked. The team will not be held responsible for the patients on the waitlist during this period. If a bed on Ward 5A is unavailable, use of a general ward bed may be considered, as per the CAMHS [Crisis Connect Entry Protocol](#).

Admission decisions are communicated to the referrer by the CCC CNC (or proxy) within 60 minutes of the referral being received. If it is decided to admit the child, the CCC CNC (or proxy) communicates this decision to the referrer and provides a welcome pack to the referrer to provide to the child and family.

If it is determined that an admission is not required, the CCC CNC (or proxy) communicates this decision to the referrer. All escalations regarding the outcome of a referral from Community CAMHS should be via a Consultant to Consultant discussion.

All bed requests to be entered into WEBPSOLIS as Ward 5A Bed Request by the CCC CNC (or proxy). Any subsequent correspondence and communication with referrer to be entered under the *CAMHS Crisis Connect* Program.

Planned Admission Requests

The Ward 5A Consultant Psychiatrist is responsible for the decision to admit to Ward 5A for a planned admission.

The following information is required:

- SS CD Initial Assessment or more recent comprehensive assessment (in ISOBAR format)
- SS CD Risk Assessment
- Medical Clearance
- Rationale for admission
- Primary goals of admission
- Social Situation Details

All bed requests to be entered into WEBPSOLIS as Ward 5A Bed Request by the CCC CNC (or proxy). Any subsequent correspondence and communication with referrer to be entered under the CAMHS Crisis Connect Program.

The details of the bed request should also be entered onto EBM.

For all planned admissions a Preadmission Planning Meeting must occur. The following timeslots are available for Planned Admission Case Conferences:

- Tuesday – 1200 & 1230.

Invitees to the Preadmission Planning Meeting include:

- Ward 5A Psychiatrist
- Referrer
- CCC Clinical Nurse Consultant (or proxy)
- Ward 5A Clinical Nurse Specialist

Preadmission Meeting

Prior to transfer to the CAMHS Inpatient Service a preadmission meeting is to be held.

The purpose of the meeting is to develop a plan which includes communication of goals, risks and management, allocation to ward areas (including bathrooms) and identification of clinical observation levels. This must be documented on the CAMHS Inpatient Service Preadmission Plan. The plan must be documented in the medical record and included in a comprehensive handover.

The preadmission meeting is to be attended by:

- Clinical Nurse (Shift Coordinator),
- Allocated Nurse,
- CCC ,(for internal referrals the CCC clinician who completed the referral assessment is required to attend the pre admission meeting)
- Psychiatry Registrar; and
- Consultant Psychiatrist and Allied Health Staff where appropriate and available.

For young people who are being admitted internally via PCH Emergency Department whom have had an admission within the previous 12 weeks transfer to Ward 5A can occur following receipt of a cRAMP and detailed preadmission plan.

For those patients who are identified as suitable for Nickoll Ward the CCC CNS will make contact with the admitting nurse at Nickoll Ward via the Shift Co-ordinator mobile 0499 834 663 to conduct the pre-admission meeting. Please refer to the CAMHS [Nickoll Ward Transport Procedure](#), should patients require transfer from Perth Children's Hospital Emergency Department to Nickoll Ward.

After the preadmission meeting, the CAMHS Inpatient Service Preadmission Plan must be signed and a copy of the Pre-Admission Plan will be emailed to Ward 5A or

Nickoll Ward via CAMHSIPUPreAdmissionPlan@health.wa.gov.au. HE numbers are to be used for signing purposes.

Outcomes of preadmission meetings must be recorded in the CAMHS Inpatient Service proforma. This form must be placed in the medical record and relevant details must be entered onto PSOLIS.

Patient Registration

The Ward Clerk must:

- Greet the child and family.
- Complete the process of identification by using the three (3) core approved patient identifiers to identify the child;
- Obtain all the demographic information required to register a patient upon their arrival to the unit;
- Print the patient identification band and labels;
- Prepare the patient file;
- Prepare the registration form;
- Confirm that family and General Practitioner (GP) details are accurate on WebPas;
 - If the patient does not have a GP recorded, the Ward Clerk must recommend that the family register with a GP within 24 hours.
 - This recommendation should be documented in the medical record. It is the responsibility of the family to ensure that their child registers with a GP.
- Identify if the patient is a private patient; and
- Confirm where the patient is admitted and add the bed number to WebPas.

If the Ward Clerk is not available (i.e. out of hours and on weekends) these tasks may be completed by nursing staff.

For further information please refer to CAHS [Patient Identification Protocol](#).

Welcome to Ward Meeting and Start of Assessments

The allocated Nurse and Registrar (or PCH on-call Registrar) must meet with the patient and family in the consult room prior to entry to Ward 5A or Nickoll Ward.

The allocated clinician(s) must ask:

- If the child would like to nominate an adult to advocate for their rights and interests (MHA s273(1)); and
- If the child understands the effect of making the nomination (MHA s 273(2)).

If the child presents without a family member, the allocated clinician(s) must ask:

- If the child has a family member (MHA s 296(2)); and
- Where the child has been assessed as having capacity (MHA ss 14(1), 15(1)), if the child consents to the family member being provided with information and being involved in their treatment or care (MHA ss 275(1), 285(1)).
- The allocated clinicians must ensure that a record of the answers to the questions asked are documented (MHA ss 275(1), 296(3)).

Patient and carer rights information must be provided, including any additional rights for involuntary patients:

- Communication
- Personal belongings
- Request to be seen by a psychiatrist.
- Right to access personal information
- Right to make a complaint

The admission packages (e.g., Welcome Pack, Clinical Care Guide etc.) must be provided and includes:

- Information regarding patient journey and the proposed Clinical Care Package
- Information about length of stay
- Information about leave
- Clear mutual expectations between patients, carers and staff
- Roles and responsibilities of patients, families and staff
- Advocacy information
- How to make complaints and provide feedback
- Rights and responsibilities
- Visiting information
- If the child is to be admitted under the MHA, this includes information from the [Mental Health Advocacy Service \(MHAS\)](#)
- Access to an Aboriginal Mental Health Worker interpreter service and multi faith chaplaincy.

The following forms must be presented and signed by the patient and family:

- Participation Agreement (MR816.18)
- Registration
- CAMHS Consent for Exchange of Information (MR510.07)

- The 'SSEN:MMH Consent for release and exchange of information', available at: [School of Special Educational Needs - Medical and Mental Health intranet page](#).

Assessments are initiated as per CAMHS policies. See '[Consent and Assessment](#)' section for more detail.

The following five (5) clinical forms and assessments are initiated at the assessment meeting:

- National Outcomes and Casemix Collection (NOCCs) – HoNOSCA,
- CGAS, FIHS, SDQ
- SSCD CAMHS Initial Assessment Form Printable and Writable
- SSCD CAMHS Risk Assessment and Management Plan Printable and Writable
- Inpatient Medication Chart (MR860.10)

Additional applicable information:

- [Form 12A – Nomination of nominated person](#)
- [Form 12C – Restriction on freedom of communication](#)

The completion of all forms at this stage is dependent on the time of admission and the capacity of the child and family. If forms have not been completed in this timeframe, the forms must be completed as soon as possible. The reason for any delay should be noted in the medical record.

Ward Orientation

The child and family must be provided with a tour of Ward 5A/ Nickoll Ward by the allocated Nurse and/or Registrar/Medical Officer. They must be oriented to the timetable, structure and shown their room. The allocated Nurse or Registrar must explain the following:

- Receiving visits - See CAMHS [Right to Communication and Receive Visitors Policy](#)
- Using service provided communication devices (e.g. communal landline telephone).
- Rules around personal electronic communication devices.
- Where personal belongings are stored and how they can be accessed or used (an inventory of belongings should be completed).
- How to participate in decisions relating to their treatment and care (e.g. through care plans, bedside clinical handover, Mutual Help community meetings).
- How to contact a Mental Health Advocate, appeal a decision, or make a complaint.
- Information regarding ward orientation must be provided:

- Verbally and in writing
- To the family and child
- A second time within 24-hours
- In a way that assists the child and family to understand:

This can include using age-appropriate language, and/or interpreter services and/or an Aboriginal Liaison Officer.

For admission to the CAMHS Inpatient Service Authorised area (e.g., Ward 5A) applicable Mental Health Commission brochures to be provided are available via [MHA 2014 resources](#). See:

- Being referred to a psychiatrist for an examination;
- Inpatient treatment orders;
- Information for voluntary patients;
- Nominated persons; and
- Receiving treatment for a mental illness.

The child and family must be asked to indicate they have received this information:

- On admission; and
- A second time within 24-hours of admission.

This must be recorded in the medical record.

Consent and Assessments

Consent to treatment must be recorded as part of the clinical assessment (this can be recorded on the Initial Assessment form).

Consent to restraint must be recorded as part of the clinical assessment and included in the patient medical records.

Belongings must be searched where indicated.

The following task(s) must be completed within **2 hours of admission** by the Registrar, PCH on-call Registrar, or allocated clinical staff:

- Risk Assessment

The following task(s) must be completed within **4 hours of admission** by the Registrar or PCH on-call Registrar:

- Initial Assessment;
- Initial Management Plan in medical record;
- Medication Chart; and
- Confirmation of phase of care

The following task(s) must be completed within **8 hours of admission** by the allocated clinical staff:

- A Treatment Support and Discharge plan

The following task(s) must be completed within **12 hours of admission** by a medical practitioner:

- A physical health assessment on admission (MHA s 241)

The following task(s) must be completed within **24 hours of admission** by a Consultant Psychiatrist:

- Review of a new admission

The allocated Nurse must complete the NOCCs. The forms must be provided to the Ward Clerk to enter into PSOLIS. The Ward Clerk must have received PSOLIS training with regards to entering these forms.

The allocated Nurse must complete a drug and alcohol assessment using the ASSIST tool and the [Fagerstrom test](#).

Documentation

- Completed MHA forms must be filed in the child's medical record.
- The patient's legal status under the status expires, must be added to the 'Legal Status Lite' function in PSOLIS.

All mandatory notifications made in relation to involuntary patients must be documented in the medical record.

The Admission Checklist, including part two, for involuntary patients must:

- Be completed by staff; and
- Signed by the child and family when they enter the service.

The following must also be documented in the child's medical record:

- All treatment decisions, including reviews of admission; and
- Any communication with the team, child, family and other relevant parties.

References and related external legislation, policies, and guidelines

[Mental Health Act 2014](#) (WA)

[Clinicians' Practice Guide to the Mental Health Act 2014](#)

Related CAHS internal policies, procedures and guidelines (if required)

CAHS Patient Identification Protocol
CAMHS Ward 5A Model of Care
POD Allocation for CAMHS Inpatient Service - Traffic Light Assessment
CAMHS Clinical Assessment Policy
CAMHS Nickoll Ward Transport Procedure
CAMHS Assessing Capacity to Consent in Minors: A Clinical Handbook
CAMHS Consent Guideline
CAMHS Mental Health Act Legal Orders and Notifications Policy
CAMHS Patient Rights under the Mental Health Act Policy
CAMHS Physical Health Assessment and Monitoring Policy
CAMHS Risk Assessment and Management Policy
PCH Searching Patients and their Belongings
CAMHS Clinical Handover Guideline
CAMHS Inpatient Leave Policy
PCH Admission and Age of Eligibility Policy

Useful resources (including related forms)
State-wide Standardised Clinical Documentation accessible via webPSOLIS: SSCD CAMHS Initial Assessment Form SSCD CAMHS Physical Examination Form SSCD CAMHS Risk Assessment and Management Plan
Record of Admission, Pre-Admission plan, MR816.08 (available to order from Print Media Group (PMG))
Participation Agreement, MR816.18 (available from PMG)
Consent for Exchange of Information, MR510.07 (available from PMG)
Falls Risk Assessment Tool and Management Plan, MR851.04 (available from PMG)
Wound Assessment Tool, MR851.05 (available from PMG)

Inpatient Medication Chart, MR860.10 (available from PMG)
SSEN:MMH Consent for release and exchange of information and Referral to SSEN:MMH Teacher forms are available at: School of Special Educational Needs - Medical and Mental Health intranet page .
WA Mental Health Commission. Mental Health Act 2014 – Information for Referring Practitioners (includes Referral Process Flowchart page 6).
WA Mental Health Commission brochures for children and families available here: Mental Health Act 2014 brochures

This document can be made available in alternative formats on request.

File Path:	CD/25/417185		
Document Owner:	Director and Head of Department PCH CAMHS		
Reviewer / Team:	A/ Nursing Coordinator – Mental Health, A/ Coordinator of Nursing Mental Health. CAMHS Policy Team.		
Date First Issued:	November 2015	Last Reviewed:	18/12/2025
Amendment Dates:	February 2020, July 2021, Nov 2024, March 2026	Next Review Date:	11/12/2028
Approved by:	Director and HoD PCH CAMHS	Date:	25/11/2025
Endorsed by:	Director Clinical Services	Date:	18/12/2025
Aboriginal Impact Statement and Declaration (ISD): #5130		Date ISD approved:	11/12/2025
Standards Applicable:	NSQHS Standards:  Child Safe Principles: 2, 3, 4, 8		
Printed or personally saved electronic copies of this document are considered uncontrolled			
Healthy kids, healthy communities Compassion Excellence Collaboration Accountability Equity Respect			